

KEY DATA & INSIGHTS FOR PHYSICIAN FOUNDERS

Direct Primary Care Trends Report 2026

DPC Trends Report 2026

Key data and insights on the growth of
Direct Primary Care — curated for physician founders

Presented by Practice Rx

www.practicerxconsulting.com

Data sourced from Hint Health (2,700+ DPC clinicians, 1.4M+ members)

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Key Takeaways

Patients are staying. Clinicians are thriving. Employers are buying in. After nearly a decade of steady growth, Direct Primary Care has moved well beyond a niche alternative into a demonstrable force reshaping primary care.

837%

Growth in DPC Members
since 2017

555%

Growth in DPC Clinicians
since 2017

60%

DPC Memberships Funded
by Employers

49

States with a DPC Network

These figures come from Hint Health's analysis of data spanning over 2,700 DPC clinicians and 1.4 million members — the largest dataset in the DPC industry.

Demand Side Trends

Current State of U.S. Healthcare

The traditional primary care system is not meeting the moment. Long wait times, rushed visits, fragmented referrals, and rising costs have eroded patient trust. For many Americans, a meaningful relationship with a primary care clinician has become the exception rather than the rule. DPC was built to fix that.

DPC practices achieved a **Total PCPCM Performance Score of 89%** — and a **97% Contact/Access score**, near-perfect, reflecting how easy it is for DPC patients to reach their doctor when they need care.

Growth in DPC Members

DPC membership increased from 44 per 100,000 Americans in 2017 to **409 per 100,000 in 2025** — an 837% increase at an average annual growth rate of ~33%. Growth is not confined to early adopters — it reflects structural, nationwide expansion.

Year	DPC Members per 100K	% Growth from 2017
2017	44	0%
2018	56	27%
2019	73	67%
2020	103	136%
2021	146	235%
2022	186	325%
2023	278	536%
2024	343	685%
2025	409	837%

For established practices, panel growth compounds over time. By year 5 on Hint Core, the median practice has grown its panel **559%** from its launch baseline.

Employer-Sponsored DPC

Employer-sponsored memberships have grown from 18% of all DPC memberships in 2017 to **60% in 2025**. Employer-sponsored membership is now the majority of active DPC memberships — and the fastest growing segment.



States with the highest employer-sponsored DPC concentration: Minnesota (95%), North Dakota (94%), Wisconsin (93%), California (88%).

Member Demographics

The DPC member profile has remained remarkably stable despite explosive growth — a strong indicator of product-market fit. Average member age: 41–43. Gender split: ~54% female. Age distribution skews 30–64, aligned with working-age employer populations.

What DPC Clinicians Charge

Membership Type	2020 Median	2025 Median	% Change
Retail (Direct-to-Consumer)	\$75/mo	\$99/mo	+32%
Employer-Sponsored	\$55/mo	\$65/mo	+18%

Employer-sponsored DPC pricing has remained stable — a stark contrast to traditional insurance premiums, which rose 43% for employers between 2016 and 2024 (KFF). Retail DPC pricing tracks closely with median household income growth (38% vs 36%), indicating affordability has not been a barrier to growth.

Supply Side Trends

The Primary Care Workforce Crisis

The AAMC projects a shortfall of 20,200–40,400 primary care physicians by 2036. Family medicine averages ~\$281K vs. ~\$564K in orthopedics, with median med school debt of \$205K. Family medicine and pediatrics rank in the top 5 specialties for burnout. DPC directly addresses all three pressures.

After transitioning to DPC, clinicians at Anovia Health reported a 48% reduction in burnout and 68% increase in job satisfaction. Average visit time doubled from 14.9 to 29.4 minutes. (Phyx / Hint Health, 2025)

Growth in DPC Clinicians

Active DPC clinicians per 100,000 people increased **555%** from 2017 to 2025 (1.1 to 7.2 per 100K). Yet DPC membership has grown even faster (+837%), meaning patient demand is outpacing supply — a significant opportunity for new entrants.



Growth of DPC Networks

DPC networks connect independent practices with employers across geographies, handling sales, contracting, and administration. In 2019, only 21 states had a DPC network. By 2025: **49 states (98%)**.

Network affiliation materially accelerates practice growth:

Milestone	Affiliated Practices	Independent Practices
Year 1	91% panel growth	83% panel growth
Year 2.5	391% panel growth	288% panel growth
Year 3	465% panel growth	330% panel growth
Year 5	657% panel growth	380% panel growth

DPC Utilization

The typical DPC member is not in the waiting room. They are in their doctor's text thread — 93% communicated with their provider at least once via SMS, email, or phone during the year.

Visit Patterns (2025)

Annual Visits	% of Members
0 (no formal visit)	47%
1 visit	13%
2–3 visits	18%
4–6 visits	13%
7–10 visits	6%
11+ visits	3%

Nearly half of members have zero formal visits annually — yet 93% are actively communicating with their physician. Access, not visits, is the product.

Communication Patterns (2025)

SMS is the dominant communication channel, with **62% of members exchanging 11+ SMS messages per year** with their physician. Email and phone supplement this always-accessible care model. DPC eliminates the friction that drives patients to urgent care and the ER.

Looking Ahead

After nearly a decade of compounding growth, DPC is transitioning from a movement into infrastructure. What was once niche is now a practical, scalable employer strategy — and recent federal policy changes have accelerated that normalization.

Key Policy Development

Legislative changes now allow employers and employees to pair DPC memberships with HSA-eligible High Deductible Health Plans (HDHPs), removing a significant structural barrier and signaling bipartisan recognition that subscription-based primary care belongs in mainstream benefit design.

What This Means for Physician Founders

- Patient demand is outpacing clinician supply — now is the time to launch or expand
- Employer-sponsored DPC is the fastest growing membership segment — build an employer strategy
- Network affiliation drives 40%+ faster panel growth than independent practices
- DPC pricing is stable and affordable relative to income — pricing is not a barrier
- The burnout crisis in primary care creates a steady pipeline of physicians exploring DPC
- HSA-pairing legislation removes a key objection employers previously had to DPC

Launch or Grow Your DPC Practice with Practice Rx

Practice Rx helps physician founders turn data like this into action — building marketing plans, employer outreach strategies, business plans, and practice development roadmaps tailored to your market, specialty, and goals.

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